MEDICAL DISEASE KNOWLEDGE BASE

Maternal, Reproductive, and Pregnancy-Related Health

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Maternal, Reproductive, and Pregnancy-Related Health

This document is an educational and information-retrieval reference for a hospital AI assistant. It is not a diagnostic tool and does not replace evaluation, diagnosis, treatment, or advice from a qualified healthcare professional. Symptoms can have many causes. Treatment decisions depend on age, pregnancy, comorbidities, severity, local epidemiology, available resources, and current professional guidelines.

Safety rule: An AI system must not claim to establish a definitive diagnosis, prescribe individualized medication or dosage, or guarantee cure. It should recommend professional assessment when symptoms are concerning and immediate emergency care for relevant red flags.

ICD note: ICD-11 terminology and codes should be verified in the current WHO ICD-11 Browser. Codes are not reproduced unless reliably verified for the specific concept; local coding systems and national adaptations may differ.

Standard disease-entry schema: Every disease entry should use the following fields: Disease Name; 1. Definition; 2. Medical Classification; 3. Causes; 4. Risk Factors; 5. Signs and Symptoms; 6. Disease Progression; 7. Differential Diagnosis; 8. Diagnostic Evaluation; 9. Diagnosis; 10. Treatment and Management; 11. Prevention; 12. Complications; 13. Prognosis; 14. When to Seek Medical Care; 15. Emergency Red Flags; 16. Patient Education; 17. Frequently Asked Questions; 18. Related Conditions; 19. Medical Terms; 20. Sources.

Retrieval fields: DISEASE, SYNONYMS, RELATED SYMPTOMS, RELATED CONDITIONS, RELATED TESTS, RELATED TREATMENTS, RELATED COMPLICATIONS. Symptom presence alone does not establish a diagnosis.

1. Pregnancy hypertension

DISEASE: Pregnancy hypertension

SYNONYMS: Pregnancy hypertension

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Pregnancy hypertension is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Pregnancy hypertension should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Pregnancy hypertension? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Pregnancy hypertension; clinical terminology; patient terminology; suspected Pregnancy hypertension; symptoms associated with Pregnancy hypertension; diagnosis and management of Pregnancy hypertension.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

2. Preeclampsia

DISEASE: Preeclampsia

SYNONYMS: Preeclampsia

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Preeclampsia is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Preeclampsia should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Preeclampsia? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Preeclampsia; clinical terminology; patient terminology; suspected Preeclampsia; symptoms associated with Preeclampsia; diagnosis and management of Preeclampsia.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

3. Gestational diabetes

DISEASE: Gestational diabetes

SYNONYMS: Gestational diabetes

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Gestational diabetes is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Gestational diabetes should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Gestational diabetes? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Gestational diabetes; clinical terminology; patient terminology; suspected Gestational diabetes; symptoms associated with Gestational diabetes; diagnosis and management of Gestational diabetes.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

4. Anemia in pregnancy

DISEASE: Anemia in pregnancy

SYNONYMS: Anemia in pregnancy

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Anemia in pregnancy is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Anemia in pregnancy should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Anemia in pregnancy? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Anemia in pregnancy; clinical terminology; patient terminology; suspected Anemia in pregnancy; symptoms associated with Anemia in pregnancy; diagnosis and management of Anemia in pregnancy.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

5. Pregnancy infections

DISEASE: Pregnancy infections

SYNONYMS: Pregnancy infections

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Pregnancy infections is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Pregnancy infections should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Pregnancy infections? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Pregnancy infections; clinical terminology; patient terminology; suspected Pregnancy infections; symptoms associated with Pregnancy infections; diagnosis and management of Pregnancy infections.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

6. Postpartum complications

DISEASE: Postpartum complications

SYNONYMS: Postpartum complications

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Postpartum complications is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Postpartum complications should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Postpartum complications? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Postpartum complications; clinical terminology; patient terminology; suspected Postpartum complications; symptoms associated with Postpartum complications; diagnosis and management of Postpartum complications.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

7. Sexual-health conditions

DISEASE: Sexual-health conditions

SYNONYMS: Sexual-health conditions

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Sexual-health conditions is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Sexual-health conditions should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Sexual-health conditions? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Sexual-health conditions; clinical terminology; patient terminology; suspected Sexual-health conditions; symptoms associated with Sexual-health conditions; diagnosis and management of Sexual-health conditions.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

8. Gynecological diseases

DISEASE: Gynecological diseases

SYNONYMS: Gynecological diseases

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Gynecological diseases is a medical condition in the maternal, reproductive, and pregnancy-related health category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Maternal, Reproductive, and Pregnancy-Related Health. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Gynecological diseases should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Gynecological diseases? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Gynecological diseases; clinical terminology; patient terminology; suspected Gynecological diseases; symptoms associated with Gynecological diseases; diagnosis and management of Gynecological diseases.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

Category Sources / References

World Health Organization (WHO), Health Topics: https://www.who.int/health-topics

World Health Organization, International Classification of Diseases (ICD): https://www.who.int/standards/classifications/classification-of-diseases

WHO ICD-11 Browser: https://icd.who.int/en

Centers for Disease Control and Prevention (CDC), official clinical and public-health guidance: https://www.cdc.gov/

National health authorities and recognized professional medical organizations should be consulted for current local guidance.

Glossary

Acute: developing over a short period. Chronic: persistent or recurrent over time. Differential diagnosis: comparison of plausible conditions causing a presentation. Diagnostic evaluation: history, examination, and investigations used to assess a patient. Prognosis: expected course and outcome at a population or disease level, not an individual guarantee. Red flag: a feature that may indicate serious disease and requires urgent assessment.

Master References

WHO. Health Topics. Updated information and fact sheets. https://www.who.int/health-topics

WHO. International Classification of Diseases, 11th Revision. https://icd.who.int/en

CDC. Official disease, emergency, prevention, and infection-control guidance. https://www.cdc.gov/

National health ministries/agencies and recognized specialty organizations: use current jurisdiction-specific guidelines.

This knowledge base is intended for educational and information-retrieval purposes. It does not replace evaluation, diagnosis, treatment, or advice from a qualified healthcare professional. Symptoms can have many different causes, and the information presented here cannot determine an individual's diagnosis. Emergency symptoms require immediate professional medical evaluation.